

Tren Capsule 250mg

Tranexamic Acid is a cyclohexyl of aminocaproic acid. It is used systemically in certain haemorrhagic states and is useful in enhancing hemostasis when fibrinolysis contributes to bleeding.

Ingredient(s):

Each capsule contains:

Tranexamic Acid 250mg

Pharmacodynamic:

1. Tranexamic acid produces an antifibrinolytic effect by competitively inhibiting the activation of plasminogen to plasmin. It is also a weak noncompetitive inhibitor of plasmin. These properties make possible its clinical use as an antifibrinolytic in the treatment of both general and local fibrinolytic haemorrhages. It has an action mechanism similar to, but about 10 times more potent *in vitro* than that of epsilon aminocaproic acid (EACA).

Pharmacokinetics:

1. Following oral administration of 200mg Tranexamic acid, a maximum concentration of 3.9µg/ml and 6µg/ml were measured after 2 to 3 hours.
2. Tranexamic acid does not bind to serum albumin. The plasma protein binding which seems to be fully accounted for by its binding to plasminogen, appears to be negligible at therapeutic plasma levels of 5 to 10mg/L.
Possible routes of biotransformation are acetylation or deamination followed by oxidation or reduction.
3. Tranexamic acid diffuses rapidly to the joint fluid and to the synovial membrane. In the joint fluid the same concentration was obtained as in the serum. The biological half-life in the joint fluid was about 3 hours.
4. Tranexamic acid is eliminated by glomerular filtration, excretion being about 30% at 1 hour, 55% at 3 hours, and 90% at 24 hours after i.v. administration of 10mg/kg.

Indication(s):

Treatment of haemorrhage, or threatened haemorrhage associated with excessive fibrinolysis. It is also used in the management of haemophiliacs undergoing dental extractions, and in the prophylaxis of hereditary angioedema.

Dosage and Administration:

The usual adult dose is 4 to 6 capsules to be taken two to three times daily. The dosage should be adjusted according to patient's age and symptoms.

Route of Administration:

To be taken orally

Contraindication(s):

1. Patients with a history or risk of thrombosis should not be given Tranexamic acid, unless at the same time it is possible to give treatment with anticoagulants.
2. The preparation should not be given to patients with acquired disturbances of color vision. If disturbances of color vision arise during the course of treatment, the administration of the preparation should be discontinued.

Side Effect(s) / Adverse Reaction(s):

Side effects of Tranexamic acid include anorexia, hypnosia, pruritus, erythema, skin rash, and heartburn. Gastrointestinal disturbances (nausea, vomiting, diarrhoea) may occur.

Precaution(s) / Warning(s):

1. Tranexamic acid should be given in reduced doses to patient with impaired renal function.
2. Tranexamic acid therapy is not indicated in hematuria caused by diseases of the renal parenchyma. Intravascular precipitation of fibrin frequently occurs in these conditions and may aggravate the disease.
3. The risk of clotting may be increased in those with underlying thrombotic states.
4. Tranexamic acid crosses the placenta. However, healthy infants have been born to women who received tranexamic acid during pregnancy for the treatment of fibrinolytic bleeding or bleeding associated with abruptio placentae. (FDA Pregnancy Category B).
Tranexamic acid is distributed into breast milk; concentrations reach approximately 1% of the maternal plasma concentration. Therefore risk-benefit of the use of tranexamic acid in nursing mother should be considered.

Drug Interaction(s):

1. Concurrent use of oral contraceptives or estrogen-containing medication with tranexamic acid may increase the potential for thrombus formation.
2. Concurrent use of Factor IX complex with tranexamic acid may increase risk of thrombotic complication in perisurgical management of hemophilic patients. Some hematologists recommend that tranexamic acid not to be administered within 8 hours of these clotting factors concentrate.

Symptoms and Treatment for Overdosage, and Antidote(s):

Symptoms of overdosage may be nausea, vomiting, orthostatic symptoms and or hypotension. There is no specific antidote for its overdose, therefore management of the patient consists of symptomatic and supportive therapy.

Shelf-life:

3 years from the date of manufacture.

Storage Condition(s):

Store at temperature below 30°C. Protect from light and moisture.

Product Description and Packing(s):

A size #2 orange cap, yellow body hard gelatin capsule.

Blister packing of 10's x 50 and 10's x 10.



Manufacturer and Product Registration Holder:
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